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Correct Answer

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Practice Exam - 1

Question 37 of 117



A 56-year-old male with a longstanding nonischemic cardiomyopathy, hypertension, diabetes and morbid obesity with a BMI of 45 presents for his third admission for acute decompensated heart failure. He is stabilized on inotropes. His BMI is considered a contraindication to transplant listing and he undergoes durable LVAD implantation. He is interested in pursuing transplant in the future. Which of the following statements accurately represents the appropriate counseling regarding weight management after LVAD implantation?

- A. Data from Intermacs demonstrates decreased survival as well as increased complications profile in obese patients undergoing LVAD implantation
-  B. Most patients lose weight after LVAD implantation
-  C. Most patients gain weight after LVAD implantation
- D. Bariatric surgery is not an option in a patient supported with an LVAD

Commentary

References

Testing Point: The Board-certified AHFTC specialist should understand the natural trajectory of weight after LVAD implantation as well as implication of obesity on LVAD outcomes

Correct Answer: C. Most patients gain weight after LVAD implantation

Rationale: Data from Intermacs as well as data from analyses of trials including Momentum and Advance have shown that most patients gain weight after LVAD implantation regardless of BMI at time of surgery.

Answer A: Registry data regarding the impact of obesity on mortality after LVAD implantation are mixed. Intermacs data suggest obese patients undergoing LVAD implantation have increased risk of device-related and medical complications, with overall survival similar to non-obese patients. In contrast, Euromacs and ISHLT registry analyses have demonstrated less favorable survival outcomes in patients with severe obesity.

Answer B: There are no data showing predictable weight loss with LVAD support.

Answer D: Bariatric surgery is an option at experienced centers for LVAD supported patients and has been reported extensively in the literature.

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